

BRIEF REPORT

How Trauma Is Represented on Social Media: Analysis of #Trauma
Content on TikTokCliodhna O'Connor¹, Giulia Brown², Julianne Debono³, Lauren Suty⁴, and Helene Joffe⁵¹ School of Psychology, University College Dublin² Department of Psychology, Carleton University³ Department of Psychology, University of Malta⁴ Department of Biology, University of California, San Diego⁵ Division of Psychology and Language Sciences, University College London

Objective: The ways that mental health concepts are represented on social media could have significant implications for lay understandings and behavior. The current article reports an analysis of how trauma is represented on TikTok, one of the world's most popular social media platforms. **Method:** Following a search for content using the hashtag #trauma, 143 videos were subjected to qualitative content analysis to characterize the profiles of their producers, intended function, and trauma-related content. **Results:** Results show that most videos were produced by young White people, who drew on their personal experience of trauma to generate confessional narratives or raise awareness of trauma. Trauma was most often attributed to childhood adversity or relationship difficulties. A diverse range of behaviors and experiences were positioned as evidence of trauma. **Conclusions:** Findings are consistent with prior suggestions that trauma's boundaries are expanding in the form of "concept creep," but also draw attention to the role of humor and irony in social media invocations of the concept. Given the current near-ubiquity of social media consumption, particularly among young people, establishing the implications of exposure to this content should be a priority for future research.

Clinical Impact Statement

Social media coverage of trauma poses both risks and opportunities for mental health. This study documents the trauma content to which people are exposed on TikTok, a social media platform particularly popular among youth. Results alert health professionals to specific messages they may wish to mitigate or reinforce in clinical work or public communications.

Keywords: trauma, social media, TikTok, concept creep, content analysis

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From its origins in describing bodily injury, the concept of trauma was expanded to index psychological harm in the mid-20th century. Posttraumatic stress disorder (PTSD) entered the *Diagnostic and Statistical Manual of Mental Disorders*, thereby becoming an official psychiatric diagnosis, in 1980; while diagnosis initially

required an exceptional and intensely distressing precipitating event, this criterion was later relaxed to incorporate even indirect exposure to subjectively difficult events (American Psychiatric Association, 1994). Bibliometric analyses of both clinical and popular English-language literature suggest that since the 1970s, the word "trauma"

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Cliodhna O'Connor  <https://orcid.org/0000-0001-8134-075X>

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Cliodhna O'Connor played a lead role in conceptualization, methodology,

project administration, supervision, writing—original draft, and writing—review and editing and an equal role in data curation, formal analysis, and investigation. Giulia Brown played a supporting role in conceptualization and methodology and an equal role in data curation, formal analysis, and investigation. Julianne Debono played a supporting role in methodology and writing—original draft and an equal role in data curation, formal analysis, and investigation. Lauren Suty played a supporting role in writing—original draft and an equal role in data curation, formal analysis, and investigation. Helene Joffe played a supporting role in conceptualization and an equal role in writing—review and editing.

Correspondence concerning this article should be addressed to Cliodhna O'Connor, School of Psychology, University College Dublin, Belfield, Dublin 4, D04 V1W8, Ireland. Email: cliodhna.oconnor1@ucd.ie

has been applied to an increasingly wide range of experiences, of progressively declining severity (Baes et al., 2023; Haslam & McGrath, 2020; Vylomova & Haslam, 2021). The term “concept creep” has been coined to reflect this extension of a concept once reserved for terrifying or life-threatening events (e.g., war, assault) to routine stressors (e.g., relationship breakdowns, career disappointments; Haslam, 2016). Understanding the dynamics of trauma’s purported concept creep is crucial since it could have both positive and negative implications, including fostering awareness and help-seeking, and/or devaluing the concept by inflating sensitivity to everyday life challenges (Haslam et al., 2020). However, minimal research reveals how the trauma concept is currently invoked in everyday, nonspecialist discourse. The present study explores how “trauma” manifests on social media via a content analysis of trauma’s representation on one of the world’s most popular social networking sites, TikTok.

A majority of individuals around the world use social media, with global social media use continuing to rise (Kepios, 2023). Social media represents a powerful vessel through which users can disseminate and consume information, express opinions and identities, and form communities that transcend physical boundaries (Yang et al., 2021). Nevertheless, persistent questions have arisen concerning the potential negative impacts of social media on mental health. Evidence confirms a correlation between higher social media use and lower psychological well-being, particularly among adolescents (Orben, 2020; Orben & Przybylski, 2019). However, responsible interpretation of such relationships requires acknowledgment of several nuances, including small effect sizes, ambiguous causality in cross-sectional data, and differentiation between the different types of social media activities and content with which people engage (Orben & Blakemore, 2023). Regarding the latter, one key dimension of social media, which poses both risks and opportunities for well-being, is content that disseminates information about mental health.

In contemporary societies, the internet is the first port of call when searching for mental health information, a trend amplified by the COVID-19 pandemic (Jacobson et al., 2020). The weakly regulated content published on social media platforms has the potential to both promote mental health awareness or stigmatization and fuel unwarranted labeling or stereotypes. Research indicates a substantial proportion of mental health-related posts on platforms such as Facebook contain medically inaccurate information (Bizzotto et al., 2023). A deeper analysis of such content can reveal nuances that go beyond concerns about factual misinformation. For instance, “pro-ana” content, which promotes disordered eating, may encourage dangerous behaviors but simultaneously represent a source of social support from similar others (Branley & Covey, 2017; O’Connor et al., 2021). Other social media analyses have revealed culturally specific patterns of stigma (Li et al., 2018), stereotype content associated with diagnoses (Robinson et al., 2019), lived experiences of mental illness beyond symptomatology (Lachmar et al., 2017), and mechanisms of peer support (Prescott et al., 2020).

The popularity of specific social media platforms vacillates across time and place. Since the unique functionality of each social media platform promotes different content, it is paramount that research keeps contemporaneous with usage trends. TikTok, a Chinese-owned video hosting service, is one of the fastest-growing platforms internationally, with over 800 million users (Statista, 2023). TikTok posts are video-based, typically under a minute long, and presented

in a user’s feed according to an algorithm that considers users’ profiles and previous patterns of viewing and engagement (i.e., likes/shares/comments) with videos on the app. Users interact through creating live/recorded videos and stories, video comments and captions, and sharing content made by others. TikTok is particularly popular among young people, with approximately half of its users aged under 24 (Curry, 2023).

Videos about mental health generate above-average levels of engagement on TikTok (Basch et al., 2022). Mental health information that is shared on the platform can reflect both personal experiences and perspectives of mental health experts or practitioners (Pretorius et al., 2022). For users, such videos may serve several purposes, including learning, identification with similar others, and self-insight or self-diagnosis (Bahnweg & Omar, 2023). A small body of research has investigated how topics related to mental health are represented on TikTok. These studies often focus on establishing the medical accuracy of information presented; for example, large proportions of content related to both autism and attention deficit hyperactivity disorder have been identified as misleading (Aragon-Guevara et al., 2023; Yeung et al., 2022). Other research has characterized the profile of content creators, for instance, showing most coverage of anxiety is produced by social media “influencers” who command large followings and present anxiety information with humorous or inspirational overtones (Zheluk et al., 2022). Further work has highlighted the role of TikTok mental health discourse in providing support and validation (Basch et al., 2022) and facilitating social connection over shared experiences of mental health issues (Greene et al., 2023; Herrick et al., 2021). However, research has also raised concerns that TikTok content may inappropriately encourage young people to adopt maladaptive behaviors/identities, for example, tic-like behaviors that mimic Tourette’s syndrome or multiple “alters” characteristic of dissociative identity disorder (Greene et al., 2023; Zea Vera et al., 2022).

In line with trauma’s rising profile in society (Haslam & McGrath, 2020), trauma has become a common topic on social media: In December 2023, TikTok’s #TraumaTok space had over 5 billion views. Despite the potentially wide-ranging repercussions of exposure to such content, no research has empirically investigated how trauma is represented on social media. This exploratory analysis aimed to establish the contexts in which trauma is invoked on TikTok and explore the meaning the concept holds on this platform. Rather than approaching trauma as a clinical or diagnostic term, the analysis prioritized exploration of how social media users themselves define and use the concept of “trauma.”

Method

Design

This study employed an inductive qualitative content analysis design to explore how trauma is represented on TikTok (Joffe & Yardley, 2003). Qualitative content analysis is an approach to classifying data according to categories that are contextually and analytically meaningful, making it well-suited for analyzing a previously unresearched space of social discourse (Neuendorf, 2017). Rather than evaluate factual accuracy or alignment with clinical/scientific concepts, the analysis’ aim was to document the meanings of trauma (i.e., interpretations or applications of the

concept) that circulate on TikTok, without imposing value judgments regarding their legitimacy.

Data Collection

To devise a search strategy, researchers first familiarized themselves with trauma content through hashtag/keyword searches via three team members' personal TikTok accounts. This generated an initial list of relevant hashtags and keywords. The hashtag #trauma was deemed an appropriate search term as it (a) represented the most viewed trauma-related hashtag; (b) retrieved videos also captioned with other related hashtags, for example, #TraumaTok; and (c) did not bias the search toward certain interpretations of trauma. To mitigate search result biases from TikTok's proprietary algorithm, a new TikTok account was created using a "clean" computer, which was factory reset to ensure no historical data of TikTok or any other app/internet use. In setting up the account, no demographic information was input except for the mandatory user age field; this was set to 19, consistent with TikTok users' typical age profile (Curry, 2023). A single search for #trauma was run on July 17, 2023, in Dublin, Ireland. Search results were ordered by relevance to mirror a typical user's experience. The first 250 videos were saved, which given the paucity of available data on the account running the search, presumably reflect videos with high engagement from other users on the platform. Due to expectations that some videos would not meet inclusion criteria, this initial sample size was selected in accordance with prior TikTok research, where samples of 100–150 videos are typical (Basch et al., 2022; Herrick et al., 2021; Yeung et al., 2022; Zea Vera et al., 2022). The 250 videos were screened by three researchers, who collectively decided whether each met prespecified inclusion criteria. In total, 107 videos were excluded: 11 because they contained imagery of a minor; 57 for using a language other than English; and 39 for irrelevant use of the word "trauma" (e.g., a musical act named "Trauma"). The 143 remaining videos were downloaded and imported into NVivo, a specialist qualitative analysis software package that allows for audiovisual data to be classified with relevant researcher-defined codes.

Data Analysis

While making screening decisions, all researchers actively made memos that noted emerging patterns and observations. After inspecting all videos and team members' memos, the lead researcher developed a coding frame (see [Supplemental Materials](#)) that classified each video according to its producer (i.e., characteristics of people shown in the video); content (i.e., information about the origins, impacts/symptoms and management of trauma); and purpose (i.e., intended function). Given the sporadic availability of and ethical challenges extracting data about content creators, coding was limited to content shared in the videos themselves, meaning information such as characters' sociodemographic characteristics was inferred based on surface appearance. Codes were not mutually exclusive, so a single video could have multiple relevant codes applied.

To promote cross-coder consistency, the first 30 videos were coded in NVivo by three researchers working collectively. A fourth lead researcher then independently coded the same 30 videos. NVivo's coding comparison query showed that most (78.6%) codes generated a Cohen's κ coefficient of $>.8$, indicating "almost perfect"

agreement (Landis & Koch, 1977). Moderate/substantial agreement ($\kappa = .41-.8$) was recorded for 18.6% of codes, whose definitions were tightened for final analysis. Two codes with poor agreement were deleted (O'Connor & Joffe, 2020). The coding frame was then applied to all videos by the original three researchers.

Once all videos were coded, code frequencies and co-occurrences were inspected to identify dominant patterns in the data. This informed an analytic narrative that summarized the content under the various code dimensions, with contextualized interpretations and illustrative examples.

Ethical Considerations

The research was approved as a low-risk study by University College Dublin. In line with relevant guidelines on social media research (Kraut et al., 2004), informed consent from content creators was not sought since data were restricted to publicly accessible TikTok videos. To protect users' privacy and intellectual property, no identifying metadata were collected and the below analytic report does not reproduce verbatim dialogue, imagery, or video links, instead paraphrasing illustrative examples. Any video containing a person likely to be a minor, as suggested by imagery, audio, or text, was removed from the data set.

Results

Producers of Videos

Nearly half (46.2%, $n = 66$) of videos showed a female-presenting person, and 37.8% ($n = 54$) of videos showed a male-presenting person, with 11.2% ($n = 16$) of nonbinary or indeterminate gender (a minority of videos showed no people, instead presenting just words or imagery such as cartoons). Most individuals in the videos appeared relatively young, with only a small minority (9.1%, $n = 13$) displaying someone seemingly over 40 years old. Most people appeared White, with only one quarter (26.6%, $n = 38$) showing a person apparently of non-White ethnicity.

Most videos (60.8%, $n = 87$) represented someone whose perspective on trauma was attributed to personal experience. One tenth (10.5%, $n = 15$) of respondents identified themselves as medical or mental health professionals, with 18.2% ($n = 26$) claiming alternative therapeutic roles (e.g., "life coach," "trauma healer"). Over one tenth (11.9%, $n = 17$) of the articles were published by people identifiable as social media influencers.

Function of Videos

Nearly half (45.5%, $n = 65$) of the videos were classed as *confessional* in nature. These involved individuals relating personal experiences of trauma. For instance, numerous videos involved disclosures of childhood experiences or parental mistreatment deemed to have a traumatizing effect (e.g., "I remember when my mom would put me on time-out for three hours in the dark").

Confessional videos often overlapped with those classed as having an *awareness-raising* function (44.1%, $n = 63$; e.g., "Today my therapist told me anxious chest pain is usually the body processing past trauma, maybe you can relate"). These videos typically sought to teach audiences how to recognize trauma or its symptoms. This often cast innocuous habits or experiences, such as difficulty with sleep ("Can't wake up to your alarm clock?") or

concentration (“Does your mind drift in conversation?”) as signs of trauma.

An additional 13.3% ($n = 19$) of the videos went beyond this to specifically *advise* users on actions they should take to manage trauma, for example, to prioritize rest or particular approaches to relationships (e.g., “If your partner is constantly asking if you are okay, this is a trauma response to growing up in an unstable environment, so give them the reassurance they need”). One tenth (9.1%, $n = 13$) of videos were classed as having a clear *promotional/marketing* function, usually for a therapeutic product offered by the creator. Approximately half ($n = 6$) of these came from individuals claiming an alternative therapeutic role such as “life coach,” with only two produced by a traditional mental health practitioner.

Humorous videos represented nearly one quarter of the sample (24.5%, $n = 35$). These typically applied the trauma label in an ironic or lighthearted fashion to minor mishaps (e.g., “Trauma is when you open the cookie tin to find sewing materials”). An additional 12.6% ($n = 18$) were classified as having an entertainment function, for instance, animated videos that deployed trauma as a narrative device.

Only six (4.2%) videos contained any element of *criticism* of trauma discourse. Some of these dismissed the concept of trauma entirely, while others reflected on potential harms of overextending the concept (“I’m so over the trauma cult characterizing everything as a trauma response”).

Content of Videos

Source of Trauma

Trauma was most commonly attributed to negative childhood experiences (44.8%, $n = 64$). Many such videos involved blame of parents for perceived mistreatment or “narcissistic” behavior (e.g., “My mother canceled my Disneyland trip the day before because my father’s girlfriend was going”). The other major perceived source of trauma involved relationship difficulties, cast as traumatizing in one fifth (19.6%, $n = 28$) of videos. A repeated trend saw users (usually female) filming themselves crying, presented as their response to a traumatic breakup (e.g., “I am processing the most traumatic breakup of my life”).

Just over one tenth (11.2%, $n = 16$) of videos invoked trauma to characterize minor challenges, such as social embarrassment (e.g., “I sat in chocolate and didn’t realize”). This was often humorous in tone.

Other less frequently mentioned sources of trauma, invoked in fewer than 10 videos apiece, included bereavement, sexual assault/abuse, mistreatment in adulthood, life-threatening situations, physical injury/illness, political/societal issues, religion/cults, parenting/childbirth, and financial difficulties.

Signs of Trauma

A wide range of experiences were identified as evidence of trauma. Relationship difficulties (24.5%, $n = 35$) were the single sign most commonly invoked (e.g., “Do you constantly push people away? This is a trauma response”). Low/disturbed mood (15.4%, $n = 22$) was the second most common (e.g., “The emotional turmoil trapped in my body is the somatic memory of being a highly

sensitive child”), followed by dependent/submissive behaviors (14%, $n = 20$) and poor self-image (13.3%, $n = 19$).

Between 5% and 10% of videos mentioned anxiety, social isolation/withdrawal, hypervigilance, body sensations, flashbacks, and dissociation. Numerous other experiences were mentioned with <5% prevalence, including problems with sleep, memory, eating, attention, and substance use.

Interventions for Trauma

Most videos did not recommend specific interventions but implicitly conveyed that trauma required individual management (sometimes under the guidance of self-identified “trauma coaches”). Under one tenth (9.8%, $n = 14$) of videos referred to psychological/psychiatric therapy. Only two (1.4%) videos referring to therapeutic interventions mentioned the diagnostic category of PTSD. The data set contained no mention of common clinical interventions for PTSD, such as cognitive behavioral therapy, eye movement desensitization and reprocessing, or psychotropic medication.

Discussion

Evidence from both clinical and cultural contexts suggests that the threshold for designating trauma has relaxed in recent decades (Baes et al., 2023; Vylomova & Haslam, 2021). However, limited evidence reveals how trauma is invoked in the everyday media that laypeople produce and consume. This study represents the first analysis of how trauma is represented on social media, in a platform widely used by contemporary youth.

Results suggest that TikTok content regarding trauma comes from a limited sector of the population, namely young White people. The underrepresentation of non-White and older people is notable since these groups may have distinct perspectives on trauma: A U.K. survey found older people and ethnic minorities classify a wider range of adversities as traumatic (O’Connor et al., 2023). These sectors’ marginalization from social media discourse may mean representations of trauma do not reflect the perspectives of populations most at risk of serious adversity (Roberts et al., 2011). However, it should be noted that the analysis was restricted to English-language content so may underestimate the participation of non-White people in trauma discourse internationally. Females slightly outnumbered males in this data set, which may be consequential given prior evidence gender is associated with systematically different types of content (Zheluk et al., 2022). For instance, self-generated imagery of one’s emotional distress was predominantly uploaded by females. Given the suggestions gender norms can impede help-seeking in men (Keohane & Richardson, 2018), a lower accessibility of templates for expressing male distress is noteworthy.

The finding that most people who spoke about trauma based their expertise on lived experience aligns with previous research showing the prominence of personal experience in TikTok coverage of mental health issues (Basch et al., 2022). Since prior evidence links personal disclosure on TikTok with higher degrees of trust, this may inflate the perceived credibility of this content (Ruangkanjanases et al., 2022). In contrast, the low prominence of social media influencers (one of the least trusted groups in society, Ipsos, 2023) in this data set contradicts a previous study suggesting influencers comprised over 85% of TikTok videos related to anxiety (Zheluk et al., 2022). This analysis suggests that, compared with other mental

health content, trauma may be distinctive in generating highly personalized narratives from ordinary social media users. It should be noted that people in the midst of dealing with serious trauma or its aftermath may be less likely to post about their experiences, which may contribute to the dilution of the concept in the data set.

The “confessional” style of videos describing personal trauma experiences often segued into efforts to raise awareness about trauma, with creators encouraging viewers to apply the trauma lens to their own lives. The minority of videos characterized as advisory in function confirmed prior suggestions that therapeutic advice is being offered on TikTok. This advice was often from people with no apparent formal qualifications and sometimes vested interests in promoting particular products (Pretorius et al., 2022). The proportion of videos produced by medical or mental health practitioners was low, leaving a vacuum into which self-proclaimed “trauma coaches” entered.

Explicit critical reflection on aspects of trauma discourse was rare. However, humor played an important role in subverting prevailing trauma narratives. A robust subgroup of videos satirized trauma’s ubiquity by applying it to patently inappropriate subjects, such as minor embarrassments. This aligns with ethnographic work showing young people commonly engage with mental health labels in playful or ironic ways (Armstrong et al., 2023). For future research exploring concept creep on social media, appreciating the ironic tone of such content is key to avoiding inaccurate inference regarding trauma’s expanding boundaries.

Negative childhood experiences and relationship difficulties accounted for the vast majority of causal attributions for trauma. These often overlapped, with many describing childhood adversities revolving around parental mistreatment or unhealthy family dynamics. The range of severity of negative childhood experiences was wide, with many examples of incidents that would not, as described in the videos, meet standard clinical or scientific thresholds for designating adverse childhood experiences (Kalmakis & Chandler, 2014). There was a notable absence of attribution of trauma to political or structural factors, such as environmental damage or inequality. Furthermore, many prototypical precursors of clinical trauma, such as violent or sexual assault (Karstoft & Armour, 2023), were infrequent in the data set. These data support previous suggestions that trauma is commonly applied to less serious adversities (Baes et al., 2023). Moreover, the expansive scope of the concept is evident in the wide range of experiences or behaviors positioned as evidence of trauma. Numerous videos recast general mental health symptoms or seemingly innocuous habits, such as difficulty responding to one’s alarm clock or finding one’s mind adrift in conversation, as signs of trauma, encouraging viewers who identified with such behaviors to review their lives to identify a historic traumatic incident. As such, the current analysis supports concerns that TikTok coverage of mental health may inappropriately encourage some young people to engage in clinically unwarranted self-diagnosis, which could potentially have self-fulfilling prophecy effects (Bahnweg & Omar, 2023; Greene et al., 2023; Zea Vera et al., 2022). Moreover, consistent with prior research (Zheluk et al., 2022), the relatively low coverage of evidence-based interventions means that once a user self-identifies as having undergone trauma, viable therapeutic pathways are not immediately apparent on this platform.

This exploratory analysis was subject to several limitations. The sample size, while typical of previous TikTok studies, represents a small proportion of all videos referencing trauma on the platform. While

the research TikTok account was created to be as demographically “empty” as possible, geolocation and age specifications were unavoidable, meaning videos may have been targeted at young adults in Ireland. Resource and ethical restrictions required the exclusion of non-English-language videos and those displaying apparent minors, which limits the generalizability of the data. Furthermore, since user account information was not available for analysis, sociodemographic characteristics were inferred based purely on visual presentations. This left space for mischaracterizing individuals’ identities (e.g., gender, ethnicity) and reduced sensitivity of distinctions (e.g., age judgments were limited to over/under 40 due to difficulty reliably classifying narrower categories). However, these impressionistic data are consistent with the experience of real TikTok users, whose feed displays videos without foregrounding information about their creators.

Finally, this descriptive analysis sheds no light on how exposure to such videos impacts TikTok users. Previous discussions of trauma’s concept creep have proposed both positive and negative repercussions, alternately encouraging help-seeking for mental health difficulties, or undermining coping by pathologizing normal life challenges (Foulkes & Andrews, 2023; Haslam et al., 2020). Moreover, many individual difference factors, including education, cognitive styles, and political orientation, may influence how people engage with health-related information on social media (Nan et al., 2022). Empirical research that establishes the range of implications of this real-world media content should be a priority for future work in this space. Further research should also explore the personal motivations of content producers (e.g., self-expression, helping others, self-promotion) and the appeal for users of engaging with trauma content on social media.

This study contributes to a wider debate about the role of social media in popularizing mental health concepts, particularly among young people. Results corroborate suggestions that in youth-focused discourse, clinical constructs such as trauma are widely applied to describe challenges of highly variable severity (Haslam & McGrath, 2020). Empirical research that unpacks the risks and opportunities posed by this social media content is vital to inform efforts to promote responsible social media engagement and oversight.

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